



DOB:
Physician:

Sex: F

Collected:
Received

Case type: Surgical Case

Accession

DIAGNOSIS

(A) RIGHT CENTRAL NECK:

METASTATIC PAPILLARY THYROID CARCINOMA IN THREE OF FIVE LYMPH NODES (3/5).

Extracapsular extension present.

(B) RIGHT INFERIOR PARATHYROID GLAND:

Parathyroid tissue, negative for tumor.

(C) TOTAL THYROID:

PAPILLARY THYROID CARCINOMA, CONVENTIONAL TYPE

Location: Right lobe

Multi-focal: No

Size = 2.9 cm

Extrathyroidal extension: Present, extending into skeletal muscle

Lymphovascular invasion: Absent

Resection Margins: Tumor about resection margin

Lymph nodes:

No lymph nodes present

1CD-0-3

Carcinoma, papillary. Thyroid 8260/3
Site: Thyroid, NOS C13.9

GROSS DESCRIPTION

(A) RIGHT CENTRAL NECK – Multiple fragments of fibroadipose tissue (3.0 x 2.0 x 0.7 cm). Specimen is dissected to reveal one possible lymph node measuring 0.4 cm in greatest dimension. Specimen is submitted entirely.

SECTION CODE: A1, one possible lymph node; A2, remainder of the specimen submitted entirely.

(B) BIOPSY RIGHT INFERIOR PARATHYROID GLAND – A piece of tan-red soft tissue (0.2 x 0.1 x 0.1 cm and 0.0028 grams). A touch prep is performed and the specimen is entirely frozen for intraoperative consultation in B.

(C) TOTAL THYROID, STITCH MARKS LEFT LOBE – Total thyroidectomy (left lobe 4.5 x 2.2 x 1.5 cm; isthmus 1.5 x 1 x 0.5 cm, and right lobe 3.4 x 2.4 x 2 cm).

The right lobe is almost completely replaced (>75%) by a firm well-circumscribed and encapsulated nodule (2.9 x 1.6 x 1.5 cm) that is partially calcified and admixed with soft friable pink-tan parenchyma. The nodule extends to the disrupted capsule.

The left lobe has a single multicystic nodule (1.2 x 1 x 1 cm) that is well-circumscribed and encapsulated with yellow tan colloid parenchyma at the inferior portion of the left lobe.

SECTION CODE: C1-C3, right lobe nodule, representatively submitted from superior, middle, inferior aspects, respectively (C3 pending decalcification); C4, isthmus, representative; C5, superior left lobe, representative, and inferior colloid nodule; C6, remaining inferior colloid nodule (entirely submitted); total 6.

CLINICAL HISTORY

Papillary thyroid carcinoma.

DOB:
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Collector
Received

Pathologist:
Case type: Surgical Case

Accession:

SNOMED CODES

T-B6000, M-80503, T-C4200

"Some tests reported here may have been developed and performance characteristics determined by
These tests have not been specifically cleared or approved by the U.S. Food and Drug Administration."

Entire report and diagnosis completed by:

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Surgical Pathology Report
File under: Pathology

Criteria	Yes	No
Diagnosis Discrepancy		
Primary Tumor Site Discrepancy		✓
HIPAA Discrepancy		✓
Prior Malignancy History		✓
Dual/Synchronous Primary Noted		✓
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	RB	Date Reviewed: 9/27/12